

Multi-Document Batch B

Synthetic medical document packet for duplicate-detection testing.

Document count: 3

Contains separate synthetic patient documents, multi-page sections, duplicate-adjacent pages, and unique content.

No real patient data. Not for clinical use.

Boundary markers in this packet are intentionally simple so downstream tools can test page grouping.

REASON FOR CONSULT

Synthetic review of recurring cough symptoms and mock medication reconciliation within a multi-document batch.

PERTINENT HISTORY

The patient in this fictional packet has a prior synthetic ED visit and a mock laboratory panel. This page is part of a generated batch, not a real medical record.

FINDINGS

No distress. Mock chest exam clear except a single placeholder wheeze statement. The exact language may repeat elsewhere in this dataset for testing.

RECOMMENDATION

Coordinate with Demo Primary Care Clinic. Schedule fictional follow-up after document-processing validation is complete.

Medication	Dose	Frequency	Comment
Lisinopril	10 mg	Daily	Mock chronic medication
Metformin	500 mg	Twice daily	Synthetic medication history
Albuterol	90 mcg	As needed	Training-inhaler example
Acetaminophen	500 mg	Every 6 hours PRN	Example OTC entry

MEDICATION RECONCILIATION NOTE

This synthetic medication list intentionally overlaps with sections from other pages but is not an exact duplicate of any full page.

Facility: Synthetic Valley Clinic**Status: Signed - synthetic****CHIEF COMPLAINT**

Shortness of breath and dry cough for three days in a synthetic adult patient.

HISTORY OF PRESENT ILLNESS

The synthetic patient reports mild chest tightness after climbing stairs, dry cough, and subjective fever. The patient denies syncope, hemoptysis, recent surgery, or known exposure to tuberculosis. Oral intake is fair. This note is generated for document-processing tests only.

VITALS

Temperature 37.8 C; heart rate 96 bpm; blood pressure 128/78 mmHg; respiratory rate 18/min; oxygen saturation 96 percent on room air; pain score 2/10.

PHYSICAL EXAM

Alert, speaking in full sentences, no acute distress. Lungs have scattered expiratory wheezes at both bases. Cardiac rhythm regular. Abdomen soft and non-tender. No lower extremity edema.

ASSESSMENT

Synthetic assessment: viral bronchitis with mild reactive airway symptoms. No real diagnosis is implied.

PLAN

Hydration, inhaler teaching using placebo training device, fever monitoring, and routine follow-up with a synthetic primary care clinic. Return precautions are included only as mock chart text.